ASSESSMENT PROTOCOL · FOR PERSONAL TRAINERS

The 5-Minute Movement Screen

Quick Assessment Protocol for Every New Client — Identify mobility restrictions, movement compensations, and injury risk before the first session begins.

6 Functional Tests | Pass / Fail Criteria | 5 Minutes Total | Programming Recommendations

6	<5	3	100%
Movement Tests	Minutes to Complete	Result Tiers	Client Ready

Used by elite trainers on EvoFit Trainer — Track, program, and re-assess with one platform

CLIENT NAME	ASSESSMENT DATE	TRAINER / COACH	RE-ASSESSMENT DATE

Instructions: Perform each test in order. Grade each movement as PASS or FAIL. Note any compensations, pain reports, or asymmetries. Complete all 6 tests before programming. Time: approximately 4–5 minutes total.

1 Overhead Squat

Ankle · Hip · Thoracic Mobility

PASS CRITERIA	FAIL CRITERIA	MODIFICATION / NOTE
Hips descend to or below parallel	Heels lift off floor	Elevate heels 1–2 inches to screen ankle mobility separately
Knees track over toes (no collapse)	Knees cave inward (valgus)	If arms fall: assess thoracic rotation
Torso stays upright (<45° forward lean)	Excessive forward torso lean	Knee valgus: assess hip abductor strength
Arms remain overhead throughout	Arms fall forward significantly	
Heels maintain floor contact	Unable to reach parallel depth	

☐ PASS ☐ FAIL Notes:

2 Single-Leg Balance

Proprioception · Ankle Stability

PASS CRITERIA	FAIL CRITERIA	MODIFICATION / NOTE
Eyes open: 30s each side without touching down	Unable to hold 30s eyes open	Start with hands on hips to remove arm compensation
Eyes closed: 15s each side stable	Repeated toe-touches or hops	Asymmetry >5s: screen for previous ankle/knee injury
Minimal trunk sway	Significant side-to-side difference (>5s)	Progress: narrow stance → eyes closed
No notable L/R asymmetry	Excessive arm-windmilling to stabilise	

☐ PASS ☐ FAIL Notes:

3 Hip Hinge (Toe Touch)

Hamstring Length · Posterior Chain

PASS CRITERIA	FAIL CRITERIA	MODIFICATION / NOTE
Fingertips reach floor or within 3 inches	Excessive spinal rounding (thoracic kyphosis)	Place dowel rod along spine: contact at head, thoracic, sacrum
Spine maintains natural S-curve	Cannot reach past mid-shin	Can't hinge without rounding: regress to box hip hinge
Hips drive back, not down	Knee hyperextension or extreme knee bend	Tight hamstrings: prioritise Romanian DL with light load
Knees soft (not locked out)	Pain or pulling in lower back	

☐ PASS ☐ FAIL Notes:

4 Push-Up Plank Hold

Shoulder Stability · Core Control

PASS CRITERIA	FAIL CRITERIA	MODIFICATION / NOTE
Hold push-up position 10s stable	Scapular winging visible	Regress to knees if unable to hold toes
Body in straight line (ankle-shoulder)	Hips drop or hike within 10s	Winging: address serratus anterior before heavy pressing
Scapulae flat, no winging	Unable to maintain without trembling/collapse	Hip sag: prioritise anterior core work in Phase 1
Hips level — no sag or piking	Shoulder pain or clicking	

☐ PASS ☐ FAIL Notes:

5 Active Straight Leg Raise

Hip Flexion · Hamstring Length

PASS CRITERIA	FAIL CRITERIA	MODIFICATION / NOTE
Raise leg to 90° hip flexion (vertical)	Leg stops below 70° without compensation	Place hand under lumbar to monitor spine position
Pelvis stays flat/neutral on floor	Pelvis tilts or rotates during raise	Below 70°: focus on passive hamstring stretching before RDLs
Down leg stays fully extended	Opposite leg bends or externally rotates	Asymmetry: note L/R separately — flag for follow-up
No spinal rotation or lower back lift	Lower back lifts from floor	

☐ PASS (Left) ☐ FAIL (Left) Right:

6 Shoulder Mobility (Apley Scratch)

Internal / External Rotation · Lat Tightness

PASS CRITERIA	FAIL CRITERIA	MODIFICATION / NOTE
Both hands touch or come within 2 inches	Hands more than 4 inches apart	Asymmetry or pain: defer overhead pressing; assess rotator cuff
Hands: one reaching over shoulder, one behind lower back	Pain or sharp discomfort at end range	Tight lats: add doorway stretching before vertical pull work
No pain, clicking, or impingement symptoms	Significant L/R asymmetry (>2 inches difference)	Both sides: test and record measurement each side
Symmetrical range both sides	Thoracic rotation compensation to reach	

PASS (Left) FAIL (Left) Right:

Step 1 — Tally Your Score

Count the number of PASS results from Page 2. Each test = 1 point. Maximum score: 6/6.

5–6 / 6
Ready to Train

Excellent movement quality. Client is cleared for progressive loading. Address any single failing test with targeted accessory work. Prioritise strength and conditioning goals.

3–4 / 6
Foundation Phase

Notable mobility or stability restrictions present. Begin with a 2–4 week corrective phase focusing on failing tests before introducing significant external load.

0–2 / 6
Corrective First

Multiple significant restrictions. Implement a dedicated corrective program (4–6 weeks). Consider physiotherapy referral if pain was reported on any test. Re-screen before loading.

Step 2 — Programming Recommendations

Overhead Squat FAIL
→ Add ankle mobility drills (90–90 stretches, wall ankle mobs) daily
→ Begin goblet squat progression with heel elevation
→ Include thoracic extension over foam roller
→ Defer barbell back squat until screen passes

Single-Leg Balance FAIL
→ Daily single-leg standing practice (2×30s each side)
→ Add BOSU balance work and single-leg RDL with light load
→ Avoid bilateral lower body loading as primary stimulus
→ Screen for previous ankle sprains or knee injuries

Hip Hinge FAIL
→ Teach hip hinge pattern using wall or box as tactile cue
→ Begin Romanian DL with light dumbbells (focus on form)
→ Add hamstring flexibility work: lying stretch, PNF
→ Hold deadlift variations until hinge pattern grooves correctly

Push-Up Plank FAIL
→ Prioritise anterior core: dead bugs, hollow holds, ab wheel
→ Address scapular winging with serratus wall slides
→ Regress pressing to incline push-up until plank is stable
→ Include band pull-aparts to improve shoulder stability

Re-Assessment Schedule

2 wk	4 wk	8 wk	12 wk
Corrective Phase Check	Formal Re-Screen	Progress Review	Full Re-Assessment

TRACK EVERY ASSESSMENT IN ONE PLACE

Run EvoFit Trainer for Your Entire Client Roster

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